Data Protection and Confidentiality Policy

Girl Pride Africa Kenya (GPAK): how personal and health data is collected, used, stored, shared and destroyed across all programme areas

1 Purpose and scope

This policy sets out how Girl Pride Africa Kenya (GPAK) collects, uses, stores, shares and destroys personal data, including the sensitive health, HIV status, pregnancy, sexual and reproductive health, violence and mental health data that GPAK's programmes handle. It applies to all GPAK staff, the Advisory Committee, volunteers, mentor mothers, peer support group facilitators, community volunteers, interns, consultants, contractors and partners who see or handle participant data, across all five programme areas (psychosocial support and mental health; health and HIV referrals; education and school re-entry; livelihoods and agriculture; family strengthening and parenting) and every project GPAK delivers, whichever funder supports it.

Every person covered by this policy signs the confidentiality undertaking at Annex A before they have any contact with a participant or her data.

2 Legal basis

GPAK processes personal data, including health data, under the Data Protection Act, No. 24 of 2019 and the Data Protection (General) Regulations, 2021. Under the Data Protection (Registration of Data Controllers and Data Processors) Regulations, 2021, an entity that processes health data falls within a mandatory registration category regardless of its size or turnover.

Policy commitment. GPAK registers with the Office of the Data Protection Commissioner (ODPC) as a data controller and keeps its registration current. The Director is responsible for making and renewing this registration and for keeping evidence of it on file.

GPAK is also guided by the Children Act, No. 29 of 2022 (children's data and consent), the HIV and AIDS Prevention and Control Act, No. 14 of 2006 (HIV testing and the confidentiality of HIV results, applied by health facilities, not by GPAK), and the Public Benefit Organizations Act, 2013 (organisational accountability). Where facility staff test a participant for HIV, the facility applies the consent and confidentiality rules in the HIV and AIDS Prevention and Control Act; GPAK does not test participants and only receives a result with the participant's separate consent to facility verification (form F4).

3 The eight data protection principles, applied at GPAK

Principle	What it means for GPAK
Lawfulness, fairness and transparency	GPAK collects data only with a lawful basis (consent or assent) and tells each participant, in her own language, what is collected, why, and who may see it.
Purpose limitation	Data collected to plan and follow up a participant's support is used for that purpose. It is not reused for an unrelated purpose without fresh consent.
Data minimisation	GPAK collects only what a support plan, referral or indicator needs (see section 5). Staff do not add extra questions out of curiosity.
Accuracy	Mentor mothers and community volunteers report changes (new contact details, a closed referral) promptly so the Programs Officer can correct the register.
Storage limitation	Data is kept only for the retention periods in section 11, then destroyed.
Integrity and confidentiality	Paper records are locked away and electronic records are password-protected and, where they include the identity key, encrypted (sections 7 to 9).

Principle	What it means for GPAK
Accountability	The Director is accountable for this policy; the Public Health and Research Officer, as MEAL lead, checks that it is followed (section 16).
Data subject participation	A participant may ask to see, correct, object to, or have deleted, her own data at any time (section 10).

4 Special categories of data GPAK handles

Across its five programme areas, GPAK handles these special categories of personal data:

- HIV status and HIV-related clinical information (received only with consent to facility verification, form F4)
- Pregnancy, antenatal, postnatal and child health information
- Sexual and reproductive health information, including family planning choices
- Experience of violence, including sexual violence
- Mental health and psychosocial wellbeing information
- Children's data (participants aged 13 to 17, and their children)

GPAK treats all of these as sensitive. Staff, mentor mothers and volunteers do not discuss a participant's HIV status, pregnancy or violence experience except as needed to deliver or record her support, and never in a public or group setting.

5 Lawful basis and consent

GPAK relies on informed consent or assent as its lawful basis for processing participant data. The applicable form depends on age and status, set out in full in PRG-02 (Participant Forms Pack):

- **F2, Consent form**: her own written informed consent, for participants aged 18 to 23, and for participants under 18 who are already a mother or married (recorded as a mature minor with the Safeguarding Focal Point's sign-off).
- **F3, Assent form**: for participants aged 13 to 17 who are not a mother (for example a pregnant girl), her own written assent plus a parent or guardian's written consent. Where involving the caregiver would expose her to harm, the Safeguarding Focal Point documents a best-interests decision and consults the Sub-County Children's Officer before she is enrolled in support.
- **F4, Consent to facility verification**: a separate, specific consent before GPAK asks a health facility, school or other service provider to confirm whether a referred service was received. A participant may give F2 or F3 consent to receive GPAK's support without giving F4 consent to verification; without F4, her record is followed up through her own report and a mentor mother's or Programs Officer's observation only.

Facility HIV testing itself is consented to at the facility, under the HIV and AIDS Prevention and Control Act 2006 and national guidance; GPAK does not seek or record that consent.

A participant may withdraw consent at any time. Withdrawal is recorded in the consent withdrawal log (MEL-03) and GPAK stops the affected processing from that date, while keeping the minimum record needed to show that consent was withdrawn and when.

6 Data minimisation: what GPAK collects, by programme area

Every programme area works from the same case record model: a referral and intake screening (F1), a needs assessment (F6), and an individual support plan (F7), all coded to the participant, never to her name. Beyond that shared core, each programme area collects only what it needs, and deliberately leaves out clinical or personal detail it does not need.

Programme area	GPAK collects	GPAK deliberately does not collect

Programme area	GPAK collects	GPAK deliberately does not collect
Psychosocial support and mental health	Referral and session dates, type of support (individual counselling, peer group, mentorship), a self-reported wellbeing rating, whether a mental health referral was completed, whether a protection concern was raised and resolved safely	Clinical diagnoses, counselling session notes beyond what the support plan needs, or details of a violence incident beyond what the safeguarding and referral pathway needs
Health and HIV referrals	Referral type, provider code, appointment date, escort need, outcome and verification source; HIV status only as a yes or no data point, and only from facility verification with F4 consent, participant disclosure, or her own report	A participant's HIV status inferred, assumed or recorded from her programme mix, appearance or behaviour; clinical detail such as a specific viral load figure, which is a facility record, not a GPAK one, unless it is the indicator itself and comes with F4 consent
Education and school re-entry	Re-entry date, term-by-term attendance status, whether an exam was registered for and completed, and the type of support provided (fees, supplies, tutoring, childcare)	Exam grades or full academic records, or a school's internal disciplinary records
Livelihoods and agriculture	Training or pathway area, completion status, whether an income-generating activity started, and a household stability assessment status	Bank or savings-group account details beyond what a stipend or starter kit payment needs, or a household's full financial history
Family strengthening and parenting	Session type and date, caregiver and participant attendance, reunification status and a home-environment review status	Personal data about family members who are not the participant's caregiver or child, beyond a caregiver contact recorded with consent

7 Participant coding and the identity key

Every routine record uses a participant code, never a name: format `GP-HB-0001` (county codes HB Homa Bay, SY Siaya, KS Kisumu, MG Migori), and `GP-HB-0001-C1` for a linked child. Codes are assigned in order at enrolment and are never reused.

The identity key (code to name, phone number, safe contact method and location) is the only place where a code is linked to an identifiable person. It is held separately from all case, referral and milestone records, in the restricted registers (MEL-03), either as a locked paper register or an encrypted, password-protected file. Access to the identity key is limited to the Programs Officer, the Public Health and Research Officer (MEAL lead) and the Safeguarding Focal Point. No other role, including mentor mothers, peer support group facilitators and community volunteers, has access to the identity key; they work from the participant code and her contact details as shared directly with them for their own follow-up.

HIV status is never written on any document, register or file that leaves the locked identity key or the coded facility-verification record with F4 consent.

8 Paper records

- All coded paper registers and the identity key are kept in a locked, fire-resistant cabinet at the GPAK office.
- Key holders are the Director, the Programs Officer and the Public Health and Research Officer. A spare key is held by the Safeguarding Focal Point for emergencies.
- Every time a file is taken from the cabinet, the person signs it out on the data access and sign-out log (Annex B) and signs it back in on return the same day.
- Participant files and registers are never taken home. Mentor mother and community volunteer follow-up notes that must travel between a home visit and the office are carried in a sealed folder, coded (no names), and returned to the locked cabinet the same day.
- Where a file must be transported to a facility, school or training provider for a verification visit, only the coded referral and verification slip (F8) travels, never the full participant file or the identity key.

9 Electronic records and digital case records

GPAK keeps digital case records, a participant's needs, referrals, follow-ups and milestones, in coded registers so that her journey can be followed and checked against records, not memory. These digital records are held to the same rules as paper ones:

- Participant data is held only on GPAK devices (organisational laptop, tablet or phone), never on a personal device.
- Every device holding participant data is protected by a password or PIN known only to its user, and locks automatically when idle.
- Any electronic file that includes the identity key (code linked to name, phone or location) is encrypted in addition to being password-protected.
- No participant data, including names, phone numbers, photographs or case details, is sent on a personal WhatsApp account. Coded, non-identifying coordination between staff (for example, a participant code and an appointment date) is sent only from a GPAK-registered number or through GPAK's official channel.
- Coded case and monitoring data is backed up monthly by the MEAL lead to a second, password-protected GPAK device or an encrypted cloud folder that only GPAK staff can open.
- If a GPAK device holding participant data is lost or stolen, the person who held it reports this to the Director within 24 hours, the Director changes any shared passwords, and the loss is treated as a suspected data breach under section 14.

10 WhatsApp peer circles

Where a programme uses a moderated WhatsApp group to keep peer support alive between meetings, GPAK follows these rules:

- The group is created and moderated from a GPAK phone by a trained staff member or mentor mother, never from a personal number.
- Members join by consent and may leave the group at any time without losing access to any other support.
- Group names never refer to HIV, pregnancy or any health condition.
- No member may share another member's health status, photographs, location or personal story without that member's consent; the moderator states this rule when the group is set up and repeats it if it is broken.
- No clinical advice is given in the group; a member with a clinical question is directed to her support plan contact or the relevant facility.
- The moderator removes any message that breaches these rules and reports a serious breach (for example, a disclosure of another member's HIV status) to the Safeguarding Focal Point.

11 Data subject rights

A participant, or a parent or guardian acting for a minor with her knowledge, may at any time ask GPAK to: see the data GPAK holds about her; correct data that is wrong; object to a specific use of her data; or have her data deleted, subject to the retention needs in section 12 and any active safeguarding or legal process.

She can make this request to her mentor mother, to the Programs Officer, in person at the GPAK office, or by phone on +254 725 737 867 or email to info@gpakgirls.org. The Programs Officer records the request and passes it to the MEAL lead the same day. GPAK responds within the period the Data Protection Act sets for this type of request, and in any case no later than 7 days from the request, confirming what will be done and by when.

12 Retention and destruction

Participant files are kept for 5 years after the participant's last contact with GPAK, or longer if a grant agreement requires a longer period. Financial records connected to participant support are kept for at least 7 years, in line with GPAK's financial management policy (POL-08).

At the end of the retention period, the MEAL lead destroys paper records by shredding and electronic records by secure deletion, and records the destruction in the shredding log (Annex C), noting the file reference, the date range covered, the method of destruction, the date and the person who carried it out.

13 Communication with participants

At enrolment, every participant records her safe contact preference (for example, a call at a specific time, an in-person message through her mentor mother, or an SMS with agreed wording) on her needs assessment (F6). Staff, mentor mothers and volunteers use only this preference.

SMS wording never reveals HIV status, pregnancy, or any other sensitive detail. Examples:

Acceptable wording	Not acceptable
"Hi, this is a reminder about your appointment this Thursday. Please call your mentor mother if you cannot make it."	"Reminder: your ART refill is due this Thursday."
"Please remember your check-in this week."	"Don't forget your antenatal or HIV clinic visit."
"Your mentor mother would like to see you this week, at a time that works for you."	A voicemail naming a clinic, a medicine or a diagnosis

Staff, mentor mothers and volunteers never leave a voicemail that mentions HIV, pregnancy, a related medicine or a related clinic name. Where a participant shares a phone with a family member, staff confirm this at enrolment and use the participant's stated safe contact method only; no sensitive message is sent to a shared line without her explicit agreement.

14 Sharing data

- **With health facilities, schools or training providers**: only the coded information needed for a specific referral or verification, and only where the participant has given F4 consent to that verification. GPAK does not share the identity key with these partners; they already hold the participant's identity through their own records.
- **With funders**: only aggregated and anonymised data (counts, percentages, disaggregated by age band, county and support received, never by name or code that could identify one person in a small group). Case examples shared in reports are written so that no participant can be identified.
- **With authorities**: identifiable data is shared with the Sub-County Children's Officer, police or another authority only when the law requires it, such as a statutory child-protection report, and only the minimum needed for that report.
- **No identifiable data leaves Kenya.** Only aggregated, anonymised data is sent to a funder outside Kenya, so no identifiable participant data crosses a border and no cross-border data transfer assessment is needed for participant records.

15 Data breach procedure

1. **Contain**: whoever discovers a suspected breach (lost file, lost device, wrong recipient, unauthorised access) tells the Director and the MEAL lead immediately, and takes any step available to limit further exposure (for example, changing a password or recovering a misplaced file).
2. **Assess**: the MEAL lead and the Director assess within 24 hours what data was involved, how many participants are affected, and whether there is a real risk of harm (for example, exposure of HIV status, location or contact details).
3. **Notify the ODPC**: where the assessment shows a real risk of harm, the Director notifies the Office of the Data Protection Commissioner within 72 hours of GPAK becoming aware of the breach, as required by the Data Protection Act and the Data Protection (General) Regulations, 2021.

4. **Notify affected persons**: where the breach creates a real risk of harm to a participant, GPAK tells her (or her parent or guardian, where safe to do so) what happened and what GPAK is doing about it.
5. **Notify the funder**: the Director notifies any funder whose programme is affected within 48 hours of GPAK becoming aware of a breach, or sooner if the grant agreement requires, without sharing identifying details in the notification itself.
6. **Record**: every suspected and confirmed breach, however small, is entered in the data breach log (MEL-03) using the data breach report form (Annex D).

16 Data protection impact assessment

Data flow	Risk	Control
Needs assessment and support plan (F6, F7) record HIV status, pregnancy, violence and mental health information	Sensitive data exposed if the paper file is seen by an unauthorised person	Coded forms only; identity key held separately; locked cabinet; sign-out log (section 8)
Referral and verification slip (F8) travels between GPAK and a facility, school or training provider	A slip lost in transit, or read by someone at the partner who does not need it	Coded, not named; sealed folder; only F4-consented cases travel; partner staff bound by their own confidentiality duties
A facility, school or training provider confirms a referral outcome under F4 consent	The partner discloses more detail than needed, or discloses to GPAK staff who do not need it	The relevant referral and verification agreement limits what is confirmed to attendance and outcome status, not clinical or personal detail; only the Programs Officer or MEAL lead receives it
Mentor mother or community volunteer follow-up notes on a home visit	Notes reveal HIV status or other sensitive information in a shared or public setting	Notes are coded; mentor mothers and volunteers do not hold the identity key; monthly supervision checks note content
Moderated WhatsApp peer circle	A member's health status or story is shared in the group without consent	Group rules stated at set-up (section 10); GPAK-moderated from a GPAK phone; breaching messages removed and serious breaches reported to the SFP
Discreet SMS reminders sent to participants	A message reveals HIV status or pregnancy if worded carelessly or read by someone else	Approved wording only (section 13); participant's own stated safe contact method used
Monthly data-quality checks and reporting to funders	Small group sizes make an individual identifiable in a percentage or count	Small groups reported as counts, not percentages, where the group is below 10 (MEL-01); only aggregated data leaves GPAK
Backups of coded case and monitoring data	A backup copy is not protected to the same standard as the original	Password-protected, GPAK-only device or encrypted cloud folder; monthly backup by the MEAL lead (section 9)

17 Roles and responsibilities

Role	Data protection responsibility
Director	Accountable for this policy, for ODPC registration and its renewal, for approving the response to a data subject request that raises a policy question, and for notifying the ODPC and affected funders of a breach.
Public Health and Research Officer (MEAL lead)	Day-to-day custodian of the identity key together with the Programs Officer and the Safeguarding Focal Point; runs monthly data-quality checks; keeps the sign-out, breach and shredding logs up to date; first point of contact for a data subject request.
Programs Officer	Explains this policy to each participant at enrolment; obtains F2, F3 and F4 consent correctly; keeps daily records coded; reports any suspected breach immediately.

Role	Data protection responsibility
Safeguarding Focal Point	Holds identity-key access for safeguarding purposes; signs off mature-minor consent; is consulted on any data-sharing decision involving a safeguarding concern.
Mentor mothers, peer support group facilitators and community volunteers	Handle only coded information; do not hold the identity key; sign the confidentiality undertaking; moderate WhatsApp peer circles according to section 10; report a suspected breach to the Programs Officer immediately.
Everyone covered by this policy	Signs the confidentiality undertaking (Annex A) before any contact with participants or their data, and completes confidentiality and data protection training before first contact.

Approval and review

Item	Detail
Policy owner	Director
Approved by	Advisory Committee
Applies to	All GPAK staff, Advisory Committee members, volunteers, mentor mothers, peer support group facilitators, community volunteers, interns, consultants, contractors and partners working on GPAK activities
Review	Every two years, or earlier if the law, a funder requirement or an incident review shows that a change is needed.

Public copy. Internal forms, annexes and signature pages are kept on file by GPAK and are available on request.